## Gungahlin Mental Health Team

ACT Government | Canberra Health Services

Gungahlin Community Health Centre, 57 Ernest Cavanagh St, Gungahlin, ACT 2912

Phone: (02) 5124 1146
Fax: (02) 6207 6785

**FACSIMILE TRANSMISSION**

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**TO:** Dr Gungahlin Family Healthcare

**DATE:** 18/01/2021

**FAX NUMBER:** 0262418550

**NO. OF PAGES** (INCLUDING COVER PAGE):

**DATE FAXED:** 18/01/2021

**FROM:** Gungahlin Mental Health Team

**URGENT:** NO **CONFIDENTIAL:** YES

Dear Dr Gungahlin Family Healthcare,

**RE: Giles Cullity DOB: 22/07/1993**

**DIAGNOSIS:**
**ICD10-AM Diagnosis**

| Code | Description | Start Date | Diagnosis Type |
|---|---|---|---|
| F25.1 | Schizoaffective disorder, depressive type | 18/01/2021 11:28:00 | Reported Problem |

**MEDICATIONS:**
**Current Medications**

| Current Medication | Strength | Dosage | Frequency |
|---|---|---|---|
| Clozaril Tablets | 100 mg | 300mg | nocte |
| Abilify Maintena Powder for prolonged release injection | 300 mg (+ solv) | 300mg | monthly |
| Blooms the Chemist Venlafaxine XR Modified release capsules | 150 mg | 150mg | mane |

**ASSESSMENT:**

Dear Doctors at Gungahlin family healthcare. Thank you for your ongoing care of Mr Giles Cullity a lovely 27 year old gentleman who is new to ACT and the Gungahlin Mental Health Services. He has the aforementioned diagnoses and medication regime transferred from South Australia. He is well supported by his family including his father who is a philosophy professor. Giles likes weight lifting and cars.

Today was a short appointment to clarify medication regime and introduce ourselves. Giles has been on Clozapine for some time and is travelling well on the regime. He is also on Aripiprazole depot and denies any side effects of either. He has some intermittent lethargy which can be difficult to manage at times. This lethargy is increasing in frequency up to three times per week. We discuss the importance of hydration given his history of increased creatine in the blood tests. We will continue to monitor this although we also recommend him making an appointment with yourself.

Although he denies the diagnosis of schizophrenia he does believe he has bipolar affective disorder and was honest about his forensic history when untreated. He is otherwise feeling physically well and has some goals for the future including obtaining some vocational work and settling into Canberra.

We will make a more thorough introductory appointment within the next few weeks and keep you updated. Thank you so much

MSE:

A: Caucasian short dark hair well built muscular young fellow casual sports dress
B: calm, pleasant, good eye contact
S: NAD
M: subjectively euthymic, objectively congruent with a restricted affect
TS/F: generally linear
TC: some grandiose obsessional themes about developing a program wich combines windows explorer and the internet despite no formal qualifications. Also some obsession with exercise and "bigorexia" termed by his father.
P: Not observed responding to non-apparent stimuli
C: alert
I: does not believe he has schizophrenia and thinks he has BPAD or cyclothymia.
J: intact

R: significant forensic risk of attempted murder when unwell and untreated. Good response to Clozapine. Minimal physical health concerns with strong focus on wellness

and being fit. Generally stable mental state and no acute thoughts of harm to self or others.

Impression:

Schizophrenia with co-morbid episodes of elevated and depressed mood, outside of psychosis, therefore diagnosis confirmed of schizoaffetive disorder.

**RECOMMENDATION AND PLAN**:

1. Please see the attached hand-over from South Australian Health Services
2. New 1st appointment
3. Continue Clozapine
4. Recovery focussed goal orientated supports - gym/vocational etc.

Regards,

Dr Arjun Khaira

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**TO:** Dr Randhir Ramchander

**DATE:** 24/02/2021

**FAX NUMBER:** 0262418550

**NO. OF PAGES** (INCLUDING COVER PAGE):

**DATE FAXED:** 24/02/2021

**FROM:** Gungahlin Mental Health Team

**URGENT:** NO **CONFIDENTIAL:** YES

Dear Dr Randhir Ramchander,

**RE: Giles Cullity DOB: 22/07/1993**

**DIAGNOSIS:**
**ICD10-AM Diagnosis**

| Code | Description | Start Date | Diagnosis Type |
|---|---|---|---|
| F25.1 | Schizoaffective disorder, depressive type | 18/01/2021 11:28:00 | Final |

**MEDICATIONS:**
**Current Medications**

| Current Medication | Strength | Dosage | Frequency |
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**ASSESSMENT:**

Dear Dr Ramchander. Thank you for your ongoing care of Mr Cullity, a lovely 27 yo gentleman with the aforementioned diagnoses and medication regimen. He arrived for a more thorough psychiatric assessment with his long-term community mental health team at Gungahlin. We were accompanied by student nurse Courtney. Please see attached collateral from South Australia as well.

Giles lives with his parents and younger brother who has mod-severe ASD. He does not work, and has recently moved from South Australia for his dad's work (Philosophy professor). His mum is a retired music teacher. Giles really likes exercise and getting as muscular/fit as possible, he also really likes reading and cars. He has recently bought a new bike which is very swish, but does not have a licence to drive because he says it is not very useful. He is not very social and does not have many friends, preferring to spend time reading, with his family, or working on his projects. He has a special interest in IT and wants to make a new world changing program. He is also interested in biology and sciences in general, and is very curious about how the brain works, how medications work etc.

Physically Giles is very fit. He still worries about some abdominal adiposity which objectively is minimal. He has asked about reducing or ceasing clozapine to reduce this, but after explaining the risks reluctantly agreed that it is not a good idea at present. Apart from this, he also talk about a lack of sensation in his muscles when working out, and this can make exercises difficult. It is uncertain if this is some sort of proprioceptive problem or something else, but he said he will talk to you about it. Otherwise he feels fit and well. He denies any physical health problems with the clozapine, and specifically denies any cardiovascular, GIT, GU or neurological side effects with the medication regimen in general. Diet and appetite is good and he is very careful about what he eats. He sleeps very well, and denies any significant weight change. He denies any fevers, night sweats or rashes.

From a mental health perspective on the whole Giles feels well. He reports his mood to be euthymic and denies any specific stresses or anxieties. Acknowledging some difficult trauma in psychosis, he denies any PTSD symptoms or concerns. He reports a history of mood elevation where he did not sleep for many days at a time, and this was during a period of homeless.

Giles himself denies any psychotic symptoms. He does however talk at length about his project. For the last number of years, if not since high-school, Giles has been trying to work on this project of a new IT system. It would be a blend of an internet browser and a file explorer, and he spoke in depth about how it would work. Although the general

concept makes sense, given that he has no IT background and yet plans to hinge all his future on this program being successful. He has not finished past year 11 and he thinks he failed this. He plans on outsourcing this to a programmer although he has no finances apart from DSP. He does not want to get another job because it will distract him from this project. Apart from this he also spoke about starting a new language which will be more legit than english. He spoke about english being very difficult to understand for those from other backgrounds who have to learn, and his new language would simplify this and remove synonyms. For example, automobile and car would be changed to just automobile. This links back in with his program, as it would use the simplified words to organise and store automatically. He feels he has progressed on the language side of things more than the program. He wants to learn about artificial intelligence and ideally would like to be an artificial intelligence researcher. He feels this would make a big difference in the world.

Apart from this he denies any other grandiose, religious or persecutory themes. He denies any hallucinations, and feels his thinking process is normal. He is orientated and alert.

Giles has been diagnosed with schizoaffective disorder, although he thinks he likely has cyclothymia. He denies any other mental illnesses, although acknowledges that he has some restricted and very specific interests but has been trying to be more broad these days. He has been homeless and been charged with murder when unwell with psychosis although he believes he was just elevated in mood, and the person was another homeless person who was going to attack him anyways. He has also attempted suicide in the past as well. He did not want to talk about his past too much though and we agreed to let it be. In the past he has been on Risperidone and Olanzapine but cannot remember how they helped him. He also takes creatine and protein powders as supplements.

Giles is one of four sons, second born. His one brother has bipolar affective disorder, as does his uncle and his grandfather. His other brother has a moderate-severe ASD. Giles generally gets along well with his family although they can but heads at times. He enjoys talking about philosophy with his father. He wants to move out though, because his parents criticise his project work, and encourage him to find some paid work.

Born in Scotland, Giles came to Australia aged 8, and then went to school in South Australia. He reports enjoying school and high-school, but liked high-school better as it was a public school rather than a private school (his mother taught music at an elite private school). He denies feeling depressed in late high-school (despite records showing the contrary). He spoke about being too focussed on working out and did not pay attention in later years of high school, eventually dropping out after year 11. He then did a comparitive STAT test to get into university, but did not focus on the course work and quickly dropped out of university as well (mechanical engineering).

Currently supported by the DSP and NDIS, he spends a lot of his time working out, working on his project and researching. He does not socialise and never had a girlfriend although he wants to have one. He feels that no girl, or friends in general, would want to be associated with someone with a mental illness and unemployed. He clearly self-stigmatizes. He currently gets an ?OT to help with his project through NDIS but otherwise does not receive much support. His brother receives a lot more support through NDIS. He denies any financial problems and is not a gambler. He has a forensic history of attempted murder and assaulting police when very unwell. It is clear he has a lot of shame about this. He is not religious. He does not study or work. He denies any history of substance misuse. We discussed vocational work and he might be interested in

IT, but is worried about the socialising and it taking time away from his project. Giles also discussed clozapine cessation. Explained that we would prefer him to be on Clozapine for at least 5 years, and before cessation we would need further specialist and forensic opinion which he was open to. We did agree that Clozapine has long term side effects and we will work in a patient orientated way to give him the best options and service, and to empower him to live best life possible.

MSE:

A: Caucasian very muscular and strong fit fellow, average height, casually appropriate sport clothing, Beard with shaved moustache.
B: calm, friendly, approachable and decent rapport established
S: NAD
M: subjectively and objectively euthymic, with a somewhat restricted affect, but gets more reactive and animated when talking about passions
TS/F: NAD
TC: there are really only a few limited interests, and clearly very over valued ideas, approaching grandiose delusional level, about his project which he is counting on to be his future financial stability. There is also clearly some heavy fixation on his body shape and fitness. He has plans to finalise his new language.
P: denies and not responding to non-apparent stimuli
C: alert
I: poor - does not believe he has schizoaffective disorder. Does not think anything he talks about is grandiose or unrealistic or too focussed on these restricted interests. ~~J: intact - makes safe reasonable decision although historically becomes quickly impaired~~ with significant risk concerns when unwell.

R: there is a very high risk of homelessness, physical illness, mental illness and harm to self and others when unwell if he were to relapse due to historic risk factors including attempted murder. However currently he is generally well and risk is at baseline with no thoughts of harm to self and others. He is vulnerable, and there is some risk of exploitation. There is risk to physical health with his excessive concerns about body image.

Impression:

~~Giles is a 27yo male with poor insight into his very much genetically driven~~ schizoaffective disorder who remains in partial remission with ongoing significant family support and medication. He has very high risks if he were to relapse and involuntary treatment must begin very early. Despite his minimal insight, with ongoing rapport and alliance with the family, he continues to be adherent to medication and leads a very active lifestyle. Currently his symptoms are on the spectrum of over-valued ideas and restricted interests rather than a florid delusional capacity, and he does not meet criteria for body dystrophic disorder although may again for on that spectrum and has a history of somatic symptoms in the past. Of note, he was understandably guarded about a lot of his past MH history and has come to Canberra for a fresh start. He is a really lovely, likeable and intelligent young person with a lot of potential and we look forward to working with him to achieve some goals and improve QOL as much as possible.

**RECOMMENDATION AND PLAN:**

1. GP to investigate odd lack of sensation in muscles

2. Ongoing gentle therapeutic alliance and rapport building with the service
3. Gently encourage some vocational work - we discussed IT as a interest as it will help with his project
4. ?MRI and previous double check brain imaging
5. Close CL monitoring of mental state
6. Build therapeutic alliance with the family
7. Metabolic bloods due
8. Clozapine protocol

Regards,

Dr Arjun Khaira

[redacted] False, indisputable

[redacted] False, because Gie City lied/withheld information

[redacted] False/inacurrate, disputable

[redacted] Subjective - projecting their values onto Gie City's life

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**TO:** Dr Randhir Ramchander

**DATE:** 31/03/2021

**FAX NUMBER:** 0262418550

**NO. OF PAGES** (INCLUDING COVER PAGE):

**DATE FAXED:** 31/03/2021

**FROM:** Gungahlin Mental Health Team

**URGENT**: YES **CONFIDENTIAL**: YES

Dear Dr Randhir Ramchander,

**RE: Giles Cullity DOB: 22/07/1993**

**DIAGNOSIS:**
**ICD10-AM Diagnosis**

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**MEDICATIONS:**
**Current Medications**

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**ASSESSMENT:**

Dear Dr Ramchander

Thank you for your ongoing care of Mr Cullity.

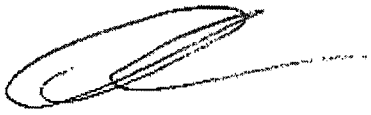
He has an elevated CK on repeated blood test results, as well as a decreased eGFR. He has reported chronic sensory problems within his muscles. (RESULTS ATTACHED)

We have consulted a renal advances-trainee who does not think it is an acute renal disease requiring urgent treatment, but recommended further assessment and referral by yourself. This includes some specific markers and potentially a 24 hour collection. In discussion with consultant psychiatrist Dr Mutlu we do not think it is related to Clozapine.

We recommend follow-up with Giles with yourself to investigate and treat this further.

Please keep us informed of any progress.

Kind Regards,



Dr Arjun Khaira
Gungahlin Mental Health Team

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**TO:** Dr Randhir Ramchander

**DATE:** 09/06/2021

**FAX NUMBER:** 0262418550

**NO. OF PAGES** (INCLUDING COVER PAGE):

**DATE FAXED:** 09/06/2021

**FROM:** Gungahlin Mental Health Team

**URGENT**: NO **CONFIDENTIAL**: YES

Dear Dr Randhir Ramchander,

**RE: Giles Cullity DOB: 22/07/1993**

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**ICD10-AM Diagnosis**

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**ASSESSMENT:**

Dear Dr Ramchander

Thank you for your ongoing care of the lovely Mr Giles Cullity, a 27yo M with the aforementioned diagnoses and medication regimen. Meng (CM) and myself conducted a home visit for routine psychiatric review whilst on Clozapine.

Giles has generally been okay. Physically he is feeling well and denies any side effects of the clozapine except that he is noticing ongoing lethargy. This lethargy has been ongoing for a few years, but both him and his father agree that over the past year it has been worsening. It is associated with poor sleep, and he is a heavy snorer. He wakes 3-4x per night, and in the morning does not feel refreshed. His neck circumference is around 18inches reportedly. 2x/week he can feel so tired he does not leave his bed much. His father denies any other depressive symptoms and Giles still continues being alert, engaging and has good motivation despite the lethargy. We discussed sleep apnoea as a differential which, after some education, he agreed with. We also reviewed the supplements he uses which are unlikely to be contributing. He now consumes a plant based protein powder, creatine, as well as a multivitamin and fish oil.

From psychiatric perspective, Giles continues with the residual grandiose themes of programming to make a fantastic web/browser app. He is also working on this other project, but importantly he has branched out to learn how to drive and is thinking of doing the road-ready course which is fantastic! He has poor insight into many of the underlying delusional themes and they are unlikely to shift significantly. He is also presenting with some negative syndrome, particularly asocial, avolition and his classicaly blunted affect.

Overall however, apart from the lethargy, Giles is happy and travelling well. The lethargy is impacting his life considerably however and he feels he does not have the energy to do a lot of things, even though he wants to. He then showed us his workout space, which is very comprehensive home gym. He will have to undo it all for the move to [redacted] which they are doing in likely the first week of July. [redacted]

We had a private discussion with dad with Giles' permission, and explained the likely negative symptoms including the restricted interests which may be impacting mental state. Explained that although treatment is difficult, management is available, including treating co-morbidities. Father was thankful for our time and happy to receive more information as well.

MSE:

Giles is a very muscular build young male with some facial hair shaved moustache, well dressed in sports clothing, good eye contact with euthymic mood and a blunted affect. Linear thought with some pre-occupation/obsessional content with grandiose themes of programming which continue. Nil perceptual or gross cognitive abnormalities, with partial insight and reasonable judgement. Chronic baseline forensic risks but no acute risk concerns of harm to self or others.

Impression:

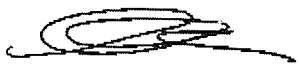
Schizophrenia in partial remission with residual chronic delusional content, and some impact of negative symptoms. Likely co-morbid OSA for treatment (large neck circumference >17in, daytime sleepiness, non-restorative sleep, loud snoring, insomnia, poor concentration).

**RECOMMENDATION AND PLAN:**

GP
- continue investigation into elevated CK (?creatine consumption?rheumatology)
- start investigation and treatment for obstructive sleep apnoea

GMHT
- written psychoeducation around negative symptoms of schizophrenia
- ongoing clozapine reviews 3-monthly
- encourage occupational independence and assistance with this

Regards,



Dr Arjun Khaira (MBBS)
Psychiatry Registrar - Advanced Trainee (Adult Psychiatry)
Gungahlin Mental Health
02 5124 1146